

4.6.4 A definite diagnosis of bone infection usually requires positive results on both histological and microbiological examinations of an aseptically obtained bone sample, but this is usually required only when the diagnosis is in doubt or determining the causative pathogen's antibiotic susceptibility is crucial. (Strong; Moderate) IDSA 2012 guidelines (Adopted)

4.6.5 Avoid using results of soft tissue or sinus tract specimens for selecting antibiotic therapy for osteomyelitis as they do not accurately reflect bone culture results. (Strong; Moderate) IDSA 2012 guidelines (Adopted)

4.6.6 When a radical resection leaves no remaining infected tissue*, we suggest prescribing antibiotic therapy for only a short duration (2–5 days). When there is persistent infected or necrotic bone, we suggest prolonged (≥ 4 weeks) antibiotic treatment. (weak, low) IDSA 2012 guidelines (Adapted)
Explanatory note: *A proximal bone histopath to be done if available to get a clear margin and confirm that no infected bone remains.

4.6.7 For specifically treating Diabetic foot osteomyelitis, we do not currently support using adjunctive treatments such as hyperbaric oxygen therapy, growth factors (including granulocyte colony stimulating factor), maggots (larvae), or topical negative pressure therapy (eg, vacuum-assisted closure). (weak, low) IDSA 2012 guidelines (Adapted)
Also refer to Table 4 and 5 in the Annexure below, for explanatory notes.

4.7. Charcot's Foot

4.7.1 Suspect acute Charcot arthropathy if there is redness, warmth, swelling or deformity (in particular, when the skin is intact), especially in the presence of peripheral neuropathy or renal failure. Think about acute Charcot arthropathy even when deformity is not present or pain is not reported. NICE clinical guideline NG19 (Adopted)

4.7.2 Refer the person with suspected charcot's foot early (within one week) to the - Diabetic Foot care centre to confirm the diagnosis, and offer non-weight-bearing treatment until definitive treatment can be started. NICE clinical guideline NG19(Adapted)

Diabetic Foot care centre:- In India, since there are no minimum standards of services offered to the diabetic foot patients, in our recommendations we have used this term to denote this facility, which may exist at the General Practitioner's office, Primary health centre, Secondary care centre or at a tertiary care centre. Preferably, the diabetic foot care centre should consist of atleast a surgeon, a physician, and an orthotist.

4.7.3 If acute Charcot arthropathy is suspected, X-ray the affected foot. Consider an MRI if the X-ray is normal but clinical suspicion still remains. NICE clinical guideline NG19 (Adopted)

4.7.4 Distinguishing the bony changes of osteomyelitis from those of the less common entity of diabetic neuro-osteoarthropathy (Charcot foot) may be particularly challenging and requires considering clinical information in conjunction with imaging. IDSA 2012 guidelines (Adopted)

4.7.5 Clinical clues supporting neuro-osteoarthropathy in this context include midfoot location and absence of a soft tissue wound, whereas those favoring osteomyelitis include presence of an overlying ulcer (especially of the forefoot or heel), either alone or superimposed on Charcot changes. IDSA 2012 guidelines (Adopted)

4.7.6 If the films show classic changes suggestive of osteomyelitis (cortical erosion, periosteal reaction, mixed lucency, and sclerosis), and if there is little likelihood of neuro-osteopathy, it is reasonable to initiate treatment for presumptive osteomyelitis, preferably after obtaining appropriate specimens for culture. IDSA 2012 guidelines (Adapted)

4.7.7 If the Diabetic Foot care centre suspects acute Charcot arthropathy, offer treatment with a non-removable off-loading device. Only consider treatment with a removable off-loading device if a non-removable device is not advisable because of the clinical or the person's circumstances. NICE clinical guideline NG19 (Adapted)

4.7.8 Do not offer bisphosphonates to treat acute Charcot arthropathy, unless as part of a clinical trial. NICE clinical guideline NG19 (Adapted)

4.7.9 Monitor the treatment of acute Charcot arthropathy using clinical assessment. This should include measuring foot–skin temperature difference and taking serial X-rays until the acute Charcot arthropathy resolves. Acute Charcot arthropathy is likely to resolve when there is a sustained temperature difference of less than two degrees centigrade between both feet and when X-ray changes show no further progression. NICE clinical guideline NG19 (Adapted)

4.7.10 People who have a foot deformity that may be the result of a previous Charcot arthropathy are at high risk of ulceration and should be cared for by the Diabetic Foot care centre. NICE clinical guideline NG19 (Adapted)

4.7.10 The Diabetic Foot care centre should care for people who have a foot deformity resulting from a previous Charcot's arthropathy as they are at high risk of ulceration. NICE clinical guideline NG19 (Adapted)

4.8: SURGICAL INTERVENTIONS AND REVASCULARIZATION

4.8.1 Consult a surgical specialist in all cases of diabetic foot infections that are moderate or severe. IWGDF 2015 (Adapted)

4.8.2 Perform urgent surgical intervention in cases of deep abscesses, compartment syndrome and necrotizing soft tissue infections. IWGDF 2015 (Adapted)

4.8.3 Debride any wound that has necrotic tissue or surrounding callus; the required procedure may range from minor to extensive. IWGDF 2015 (Adapted)

4.8.4 Perform urgent surgical intervention for most foot infections accompanied by gas in the deeper tissues, an abscess, or necrotizing fasciitis, and less urgent surgery for wounds with substantial nonviable tissue or extensive bone or joint involvement. IDSA 2012 guidelines (Adapted)

Additional note: In those with a non-severe infection, carefully observing the effectiveness of medical therapy and the demarcation line between necrotic and viable tissue before operating may be prudent.